

When is it more than tiredness?

IF YOU ARE IN DANGER NOW

If you may act on thoughts of suicide, have a plan or the means, cannot keep yourself safe, or are in immediate danger from another person: **contact emergency services, or go to the nearest emergency department, now.**

Suicidal thoughts without a plan or intention still need prompt professional help: a doctor, crisis line or urgent mental-health service, **today.**

Use a crisis or abuse service from a safe device. Tell a trusted person and do not remain alone. If it is safe, put distance between yourself and anything you could use to harm yourself. If you do not know the number, ask a hospital, a doctor or a trusted person to help you make contact. The rest of this worksheet can wait.

Private: no one needs to see this page. Tick any sign that describes you.

Signs the retreat surfaced more

- ☐ The dip has not lifted: flat most days for two weeks or more, or deepening rather than easing
- ☐ The grief remains overwhelming: disrupting sleep, eating, or my ability to cope
- ☐ Something surfaced that frightens me: a memory, a pattern of thought, a fear
- ☐ I recognised myself in Lesson 5: “not me” before the retreat, “perhaps me” now
- ☐ My rule of life keeps being knocked over: anxiety, low mood, compulsions, panic
- ☐ I have been faithful, and I am not getting better

You do not have to wait two weeks, or reach any threshold, before asking for help. Severe, rapidly worsening, stopping ordinary life, or thoughts of suicide or self-harm: seek help now.

Your step

1. Which sign, if any, describes me?

2. The right first call (the kind of help, not yet the name):

3. The objection that has held me back, and my answer:

4. My one step this week:

If my need is urgent, I will take that step today, not later in the week. If none of the signs applies: who I would call if one did.

The right first call

Titles, qualifications and how care is accessed vary by country. Most serious difficulties need more than one kind of help; a pastor is often one part, not the whole.

Need	Appropriate help	What they do
Thoughts of death, suicide, or not wanting to be alive	Prompt professional help from a doctor, crisis line or urgent mental-health service, even without a plan or intention; emergency services or the nearest emergency department if you may act, have a plan, have access to the means, or cannot stay safe	Assess immediate risk, help you remain safe, and connect you with continuing care.
Spiritual dryness, questions of faith, guidance about a decision, church-related hurt, ongoing pastoral support	A pastor or spiritual director	Prayer, Scripture, spiritual counsel, discernment, community. Not diagnosis or treatment.
Grief, relationship difficulty, life transitions, moderate anxiety or low mood, patterns you want to understand	A qualified counsellor or therapist	Talking therapy. Many are Christian or respectful of faith; ask.
Persistent low mood, loss of interest, hopelessness, sleep and appetite changes, panic, intrusive thoughts, trauma symptoms, anything affecting daily functioning	A doctor or qualified mental-health professional, according to how care is accessed in your country; a doctor especially where physical illness, medication, withdrawal or major sleep changes may be involved	Assessment, diagnosis where appropriate, and treatment, which may include therapy, medication, or both.
Abuse or coercive control, past or present	A specialist domestic-abuse or sexual-violence service, contacted from a safe device; emergency services if there is immediate danger and it is safe to call	Help you assess danger, make a safety plan, and consider housing, legal, medical or police options without pressuring you. Joint counselling or a couples' retreat may be unsafe where abuse exists.
Substance dependence	A doctor and an addiction service; a recovery community	Safe withdrawal, treatment, ongoing recovery support. If you may be physically dependent, do not stop abruptly on your own; seek medical advice.
Behavioural addiction	A counsellor with relevant experience; an accountable community	Therapy and accountability.
Scrupulosity, compulsive confession or checking, religious anxiety	A clinician experienced in OCD and scrupulosity, particularly cognitive behavioural therapy including exposure and response prevention where appropriate	Evidence-based treatment. Repeated reassurance, confession and requests for certainty can become part of the compulsion; a pastor working alongside should avoid reinforcing it.
Serious misconduct, safeguarding concerns, legal exposure	The appropriate safeguarding, legal or professional authority	Accountability, reporting, protection of others.

Choosing a counsellor

Ask about registration or recognised credentials; training relevant to what you are bringing; their approach, and why; experience with grief, trauma, burnout, anxiety, OCD, abuse, addiction or clergy care, as relevant; how they relate to faith; confidentiality and its limits; costs and cancellation; review points; and how concerns are handled.

After two or three sessions: do I feel respected, listened to, and able to raise concerns? If not, it is acceptable to change. If cost is a barrier, ask about sliding scales, charitable services, church schemes, employee assistance programmes, university training clinics and public provision.